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Bisacodyl Suppositories BP 5 mg / 10 mg

# CONLAX®-5/10

**Composition :** Each suppository contains :
Bisacodyl BP 5 mg / 10 mg
Excipients q.s.

**Product Description:** White opaque bullet shaped suppositories

**Indications :**
CONLAX is indicated for use in patients suffering from constipation.
It is also used for preparation of diagnostic procedures such as in pre- and postoperative treatment and in conditions which require defecation to be facilitated.

**Contraindications :**
CONLAX suppository is contraindicated in patients with ileus, intestinal obstruction, and acute abdominal conditions including appendicitis, acute inflammatory bowel diseases, and severe abdominal pain associated with nausea and vomiting which may be indicative of the aforementioned severe conditions.
CONLAX suppository is also contraindicated in severe dehydration and in patients with known hypersensitivity to Bisacodyl or any other component of the product.
CONLAX suppositories should not be used when anal fissures or ulcerative proctitis with mucosal damage are present.

**Dosage and Administration :**
**Short term treatment for constipation**
Adults and children over 10 years: 10mg
Children 4 - 10 years: 5mg
Children aged 10 years or younger with chronic or persistent constipation should only be treated under the guidance of a physician. Bisacodyl should not be used in children aged 4 years or younger.
**For preparation of diagnostic procedures and pre or post-operative treatment**
To be administered on the following morning for the complete evacuation of intestine.
Should be used only under medical supervision
Adults: 10 mg
Children over 4 years: 5 mg

**Pharmacodynamic properties :**
Bisacodyl is a locally acting laxative from the diphenylmethane derivatives group having a dual action. As a contact laxative, for which also antiresorptive hydragogue effects have been described, bisacodyl stimulates after hydrolysis in the large intestine, the mucosa of both the large intestine and of the rectum. Stimulation of the mucosa of the large intestine results in colonic peristalsis with promotion of accumulation of water, and consequently electrolytes, in the colonic lumen. This results in a stimulation of defecation, reduction of transit time and softening of the stool. Stimulation of the rectum causes increased motility and a feeling of rectal fullness.

**Pharmacokinetic properties**
**Absorption and Distribution:**
After oral and rectal administration, only small amounts of the drug are absorbed and are almost completely conjugated in the intestinal wall and the liver to form the inactive BHPM glucuronide.

**Metabolism and Elimination:**
The plasma elimination half-life of BHPM glucuronide was estimated to be approximately 16.5 hours. Following the administration of bisacodyl coated tablets, an average of 51.8% of the dose was recovered in the faeces as free BHPM and an average of 10.5% of the dose was recovered in the urine as BHPM glucuronide. Following the administration as a suppository, an average of 3.1% of the dose was recovered as BHPM glucuronide in the urine. Stool contained large amounts of BHPM (90% of the total excretion) in addition to small amounts of unchanged Bisacodyl.

**Preclinical safety data:**
No additional data of relevance to the prescriber.

**Undesirable effects:**
CONLAX Suppositories are suitable for most people, but, like all medicines, they can sometimes cause side effects. It may sometimes cause rectal irritation and sloughing of epithelium. It may cause abdominal discomfort such as colic or cramps. Prolonged use or over dosage may result in diarrhoea with excessive loss of water and electrolytes, especially potassium; there is also possibility of developing an atonic non-functioning colon. Hypersensitivity reaction including angioedema and anaphylactic reaction are reported rarely.

**Warning and Precaution:**
CONLAX suppositories should not be used for more than five days without investigation of the cause of constipation. Prolonged use may lead to the fluid and electrolyte imbalance, excessive loss of potassium can cause hypokalemia.
Intestinal loss of fluids can promote dehydration. Symptoms may include thirst and oliguria.
In patients suffering from fluid loss where dehydration may be harmful (renal insufficiency, elderly patients) CONLAX should be discontinued and only be restarted under medical supervision. Dizziness and or syncope have been reported in patients who have taken CONLAX. The details available for these cases suggest that the events would be consistent with defecation syncope (or syncope attributable to straining at stool), or with a vasovagal response to abdominal pain related to the constipation, and not necessarily to the administration of Bisacodyl itself.

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CONLAX Suppositories should not be used when anal fissures or ulcerative proctitis with mucosal damage are present.

**Pregnancy and lactation:**
**Fertility**
No studies on the effect on human fertility have been conducted.
**Pregnancy**
There are no adequate and well-controlled studies in pregnant women. Long experience has shown no evidence of undesirable or damaging effects during pregnancy.
**Lactation**
Clinical data show that neither the active moiety of bisacodyl (BHPM or bis-(hydroxyphenyl) pyridyl-2-methane) nor its glucuronides are excreted into the milk of healthy lactating females. Nevertheless, as with all medicines, CONLAX should not be used in pregnancy, especially the first trimester, and during breast feeding unless the expected benefit is thought to outweigh any possible risk and only on medical advice.

**Overdose :**
**Symptoms**
If high doses are taken watery stools (diarrhoea), abdominal cramps and a clinically significant loss of fluid, potassium and other electrolytes can occur.
Laxatives when taken in chronic overdose may cause chronic diarrhoea, abdominal pain, hypokalaemia, secondary hyperaldosteronism and renal calculi. Renal tubular damage, metabolic alkalosis and muscle weakness secondary to hypokalaemia have also been described in association with chronic laxative abuse.
**Treatment**
In case of overdose, immediately consult your doctor

**Drug interaction with other medicinal products and other form of interactions:**
The concomitant use of diuretics or adreno-corticosteroids may increase the risk of electrolyte imbalance if excessive doses of CONLAX are used.

**Excipients List :** Hard Fat BP(Witepsol H15)

**Shelf Life :** 2 Years (24 Months)

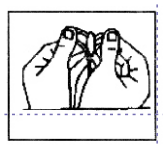
**Incompatibilities:** None known

**Storage :**
Store in a dry place, below 30°C. Protect from Light. Keep out of reach of children.

**For rectal administration only. Not to be taken orally.**

**Nature and contents of container:**
CONLAX suppositories are packed in thermo formable polyvinylchloride foil coated with Polyethylene.
Available pack sizes: 1 x 5's and 20 x 1 x 5's.

## How to use Suppository





The suppository should be inserted deeply in to the anus, if possible after having the bowels open.The suppository should be inserted deeply in to the anus, if possible after having the bowels open.

1. When practical, clean the area around the rectum with mild soap and warm water and rinse thoroughly. Gently dry by patting or blotting with toilet tissue or a soft cloth.
2. Do not open suppository if it seems soft, hold the foil wrapper under cold water or place it in refrigerator for few minutes to harden it before removing the wrapper.
3. Position the patient to lie on one side with knees bent up to their abdomen or lie on back with feet and legs held up.
4. Wash your hands and detach the suppository by holding strip upright and carefully peel the wrapper evenly down both sides. (moisten the suppository if dry)
5. Hold the suppository between index finger and thumb, locate the anus and gently insert suppository tip end first with the index finger. The suppository should be fully inserted in to rectum against the wall of rectum.
6. Hold buttocks together lying down for about 5 minutes to avoid having the suppository comes out.
7. Discard used materials and wash your hands thoroughly.

Version : 3
**Date of Revision :** 08/2020.

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Manufactured by :
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Fiber Direction

Leaflet Travel Direction